

OREGON STATE HOSPITAL

POLICY

SECTION 8: Safety, Security, Emergency Management **POLICY: 8.026**

SUBJECT: Visitors to Patients

POINT PERSON: Directory of Security

APPROVED: Sara Walker, MD  **DATE: OCTOBER 4, 2024**
Interim Superintendent

SELECT ONE: ☐ New policy ☐ Minor/technical revision of existing policy
☐ Reaffirmation of existing policy ☒ Major revision of existing policy

I. PURPOSE AND APPLICABILITY

- A. This policy and related procedures are established to facilitate safety and security for patients, staff, and visitors during in person contact, non-contact, and video visits at Oregon State Hospital (OSH).
- B. This policy applies to all staff.

II. POLICY

- A. OSH supports patient contact with family and community members in accordance with a patient's right to visitors per 42 C.F.R. § 482.13 and Oregon Administrative Rule (OAR) 309-106-0010.
- B. In person contact visitation may be permitted for patients residing on Secure Residential Treatment Facility (SRTF) units only as outlined in Procedures A "In Person Visitation Process." All other patients may utilize video visitation (Procedures B) or non-contact visitation (Procedures C) as available.
- C. All prospective visitors, unless exempt by this policy, must apply to visit OSH using the visitor application (attachment A) and must be reviewed and approved or denied per Procedures D.
- D. The following persons are exempt from the visitor application process, including the criminal records check:
 - 1. A patient's attorney recognized by the court as representing the patient, or an agent of the attorney (e.g., investigator, paralegal), given each person

- provides evidence of their representation of the patient. This includes but is not limited to state bar membership, a letter of representation from the law firm, or engagement letter from the investigation firm;
2. A court-appointed, attorney-appointed, or otherwise contracted licensed psychologist or psychiatrist whose purpose of conducting an independent evaluation of the patient and where legal authority to conduct the evaluation has been provided to OSH, such as but not limited to a court order or letter of engagement signed by the patient, guardian, judge, or patient's attorney; or
 3. A patient's legal guardian who has supplied proof of guardianship.
- E. Although a patient's attorney or clergy is not restricted to established visiting hours or location per OAR 309-106-0010, OSH will encourage attorneys and clergy to use established visitation hours and locations, so patients are not unduly disrupted.
1. Attorneys and clergy must call 2 hours in advance to set up an appointment outside of established visiting hours.
- F. An annual criminal records check must be completed for each prospective visitor aged 18 years and older unless an exception in this policy applies.
1. A current, approved visitor who meets criteria listed in the Annual Law Enforcement Data System (LEDS) must have an annual review of criminal records check.
 2. Any visitor with criminal history must have the information processed according to the standard process listed in this policy.
- G. The Director of Security may place limitations on visitation by notifying the visitor and patient in writing because they do not meet visitation criteria or based on a safety concern, security risk, or disruption to hospital operations.
- H. Security shall review patient/visitor behavioral events during visitation and determine whether a visit should be terminated, and whether there should be any restrictions to future visits.
- I. The Director of Security must review any visits requests between a patient and former staff, deny or approve the visitation based on criteria in this policy and based on considerations of safety and security to the patient, staff, and hospital.
1. The Director of Security must consult with the Superintendent to review staff history.

- J. Current staff may not visit a patient unless reviewed by the program director, the interdisciplinary treatment team (IDT), and approved by the director of security or designee.
 - a. All staff who visit a patient on non-working time must follow the outlined visitation process.
- K. When two patients living on two separate units request to visit each other, the patients must first obtain the approval of both IDTs by completing the OSH visitor application. If both patients are housed on SRTF units, in-person visitation is permitted, non-contact visitation or video visitation may also be utilized. Security must approve both patient's applications for a visit between patients to occur.
- L. Three-way visits (i.e., visitor and two patients from different areas) must be reviewed by the IDTs, Program Director(s) and approved by Security upon completion and submission of a visitor application for each person seeking to participate in the visit. If all patients are housed on a SRTF unit, in person visitation is an option; otherwise, the patients must utilize video visitation.
- M. In addition to being approved by Security, a visit with a minor must be reviewed and supported by the patient's IDT and determined to be therapeutic and safe for all involved.
 - 1. All minors must be accompanied by a pre-approved adult visitor who is a parent, legal guardian, or court-ordered escort to enter a visitation area. Visitors must provide documentation confirming that each child visiting is their child, in their legal custody or court-ordered physical custody at the time of the application.
 - 2. A minor may only visit through video or during contact visits unless an exception is approved by the program director and security leadership.
- N. Staffing exceptions are coordinated by the unit nurse manager, visitation committee, and/or IDT. Staffing expectations are outlined in Nursing protocol 2.260 and Security Protocol 5.019.
 - 1. Visitation staffing ratios must allow for maintenance of required unit staffing throughout the hospital.
- O. Video visitation is available to all patients and their approved visitors.
 - 1. Refer to Procedures B for scheduling video visitation and OSH Patient Visitation page on OWL for additional video visitation resources.
 - 2. The Visitation Committee establishes visitation hours, visitation length, and staffing requirements for video visitation.

3. Video visits will be held in a designated location on each unit determined by unit staff and applicable staffing requirements of each unit. An exception may be requested for visits that may require increased privacy for confidentiality purposes.
- P. Visitation may be supervised, limited to a specific location, or denied as set out in this policy where there is reason to believe such measures are necessary or reasonable. Limitations or denials of visitation must be documented as set out in this policy. Denials of visitation may be appealed as set out in OAR 309-106-0035.
- Q. Visitation may be limited where the unit or hospital is under a medical restriction because of a communicable disease, or where the visitor may have a communicable disease or refuses to comply with required preventative protocols. The Infection Prevention Department may recommend or require other options for visitation while the hospital is in a communicable disease situation. A visitor may be required to wear personal protective equipment, including a mask, and/or undergo health screening (i.e., temperature check and health questions) as recommended by IPEH while a medical restriction is in effect.
- R. A visitor may only bring a pet or service animal to in-person visitation if the animal has been approved in accordance with OSH policy 6.059, "Animals at OSH."
- S. Staff must ensure visitors do not bring in any property, including food items, and must comply with OSH policy 7.001, "Mail and Packages for Patients," 8.037 "Patient Property," and 4.010 "Patient Funds."
- T. If an unauthorized item is discovered in visitation:
1. If the item is illegal or criminal in nature, staff must follow OAR 309-108, applicable security protocols and actions as follows:
 - a. Notify security as necessary;
 - b. Create a chain of custody and hold the item in a secure area;
 - c. Contact law enforcement and turn over the contraband, if law enforcement is willing to accept it, and document the action taken;
 - d. If law enforcement declines to investigate further or accept the contraband, then dispose of the contraband in the presence of at least two OSH staff, and document the action taken;
 - e. Complete an incident report as outlined in OSH policy 1.003 "Incident Reporting."
 2. If it is an item that is not illegal but is not permitted inside the secure perimeter, staff must escort the visitor to the secure lobby and require the

visitor to either place the item in a locker, return the item to their car or terminate the visit depending on the item.

- U. A visitor must follow visitor guidelines during in-person and video visits or while on OSH property. For clinical and safety reasons, refusal to follow visitor guidelines will result in visits being terminated, and may result in restrictions, denial of visitations, or the loss of visiting privileges.
 - 1. Visitors must not bring items for patients to use during their visit or to take back to their unit.
 - 2. Visitors are permitted to bring only these items into the secure perimeter:
 - a. OSH locker keys;
 - b. OSH visitor badge;
 - c. Pre-disclosed and approved prescription medication;
 - d. Pre-disclosed and approved medical device(s) or equipment;
 - e. Previously disclosed and approved animal that complies with OSH policy 6.059; and
 - f. When there is a minor visitor under the age of two, the following items are allowed into the secure perimeter:
 - i. A clear baby bottle with fluids inside;
 - ii. A single baby blanket;
 - iii. Two diapers and diaper wipes; and
 - iv. One undamaged plastic toy.
 - 3. Strollers, baby carriers, car seats, and stuffed animals are not allowed in the secure perimeter.
 - 4. Any additional items must be approved on a case-by-case basis by the security shift supervisor.
- V. An OSH patient who receives care at another healthcare facility is not allowed visitors at that facility unless:
 - 1. The visit is consistent with that facility's visitation policies,
 - 2. There are extenuating circumstances to support the necessity of the visit,
 - 3. The Program Director, Chief Medical Officer, Chief of Medicine, or Social Work has communicated with that facility about OSH concerns or requirements for the visit, and
 - 4. The Director of Security or designee approves the request.
- W. The OSH Security Department must provide training to all OSH Security Department personnel at the time of hire on the provisions of this policy.
- X. OSH follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation

standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.

- Y. Staff who fail to comply with this policy or related policy attachments or protocols may be subject to disciplinary action, up to and including dismissal.

III. DEFINITIONS

- A. "Attorney" means any lawyer actively licensed by the Oregon State Bar.
- B. "Clergy" means any member of the community who is recognized by a religious authority and who has been accepted, oriented, trained, and approved by the Oregon State Hospital Spiritual Care Department. "Clinical staff" means any person employed or contracted by OSH to provide clinical services to patients.
- C. "Contraband" means any controlled substance, drugs not prescribed to the patient, drugs not prescribed or authorized by OSH, drug paraphernalia, weapons, unauthorized currency, or prohibited items.
- D. "Denied" in this policy means prohibited.
- E. "Direct Care Staff" means any health care workers, either employed directly or contracted with OSH, who work directly with patients in the hospital, including but not limited to physicians, nurse practitioner, registered nurse, nursing assistants, therapists, and technicians
- F. "Forensic commitment" or "forensic patient" is as defined in OSH policy 6.029, "Risk Review and Privileges." Forensic commitment or forensic patient means a patient committed to OSH under the following forensic jurisdictions:
1. Psychiatric Security Review Board (PSRB), which includes Guilty Except for Insanity (GEI) under Oregon Revised Statute (ORS) 161.327 and civil commitment under ORS 426.701 and 426.702.
 2. GEI-misdemeanor under ORS 161.328; or
 3. Dual Department of Corrections (DOC) jurisdiction (see ORS 179.473).
 4. "Forensic commitment" or "forensic patient" does not include civil commitment under ORS 426.130.
- G. "Legal guardian" ("guardian") for the purpose of this policy means an individual appointed by a court of law to act as guardian of an adult under ORS Chapter 125.
- H. "Minor" means any person less than 18 years of age.

- I. "OSH Property" means OSH buildings, surrounding grounds, and parking lots. Personally owned vehicles parked on OSH property are not considered part of OSH property.
- J. "Prohibited Item" means:
 - 1. Alcohol;
 - 2. Possession of drugs in the patient's room without authorization by OSH;
 - 3. Any item that reasonably could be used to escape or leave the hospital without authorization;
 - 4. Food stored in the patient room or in the patient's storage in the patient property room;
 - 5. Any item in the possession of a patient who is prohibited from possessing or using that item in their treatment care plan because it is considered detrimental to their treatment;
 - 6. Excess personal property;
 - 7. Any limited access item in the possession of a patient that exceeds the scope of the authorization or is not being supervised by OSH staff as required; or
 - 8. Any item that could pose a safety or security risk for the hospital.
- K. "Restricted" in this policy means approved with limitations.
- L. "Secure Perimeter" is defined in 309-106-0005 as a restricted high-security buildings, areas, and quads within the sally port entrances and exits at the state hospital.
- M. "Staff" includes employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at OSH.
- N. "Visitation Committee" is a group of OSH management staff working to centralize the patient visitation process and to support/encourage patient visitation while meeting the hospital's need to maintain patient safety and security.
- O. "Visitor" means an individual who is not an OSH staff member or another patient and who OSH has authorized to enter a designated visiting area in the hospital to meet with a patient.
- P. "Visit" or "visitation" means a meeting at the hospital between patient and an approved visitor who are permitted to see and talk with each other on a scheduled basis for a reasonable period of time.
- Q. "Weapon" means any item that can be used or modified to be used to harm the patient or others, including but not limited to guns, knives, improvised pointed or bladed instrument, and pepper spray.

Procedures

Procedures A In Person Visitation Process

Procedures B Video Visitation

Procedures C Visit Process

Procedures D Visitor Application

IV. ATTACHMENTS

Attachment A Visitor Application Worksheet

V. RELATED OSH POLICIES AND PROTOCOLS

1.009 Tours at OSH

4.005 Visitor Cottage Use

5.009 Staff-patient Relationships

5.018 Volunteers

5.029 Tobacco and Smoke-free Facility

6.059 Animal-assisted Interventions

7.001 Mail and Packages for Patients

7.003 Photographing or Recording Patients

7.005 Patient Rights

7.007 Visitation of Patients by Pastoral Visitors/Volunteers

7.011 Faiths, Beliefs, and Practices

7.012 Patient Responsibilities and Family Responsibilities

8.004 Criminal Offender Information Access

8.007 Identification Badges

8.037 Patient Property and Valuables: Handling and Storage

8.044 Contraband

VI. REFERENCES

42 CFR § 482.13(h).

Joint Commission Resources, Inc. (2024). *The joint commission comprehensive accreditation manual for Behavioral Health and Human Services*. (RI 01.01.03, EP 2; RI 01.06.05 EP 17, 18, 20). Oakbrook Terrace, IL: Author.

Oregon Administrative Rule §§ 309-106-0000 – 309-106-0020.

Oregon Administrative Rule §§ 943-045-0410 – 943-045-0430.

Oregon Administrative Rule §§ 943-045-0490 – 943-045-0500.

Oregon Revised Statute § 426.385.

Oregon Revised Statute § 441.414.